

Postirradiation Sclerodermatous Panniculitis

AT A GLANCE

Clinical

Women with a history of breast cancer treated by radical mastectomy and radiotherapy develop indurated erythematous plaques on the irradiated skin 1 to 6 months after radiotherapy. The clinical presentation often raises concern for cutaneous metastasis, but histopathology reveals a benign inflammatory process.

Histopathology

- Lobular panniculitis without vasculitis
- Hyaline collagen bundles in the dermis and subcutaneous septa
- Inflammatory infiltrate of lymphocytes and plasma cells involving fat lobules
- Lipophagic granulomas at the periphery of fat lobules
- Lymphoid follicles may be scattered in the septa and fat lobules

Differential Diagnosis

- **Morphea profunda:** Primarily a septal panniculitis with thickened sclerotic septa; fat lobules are typically spared.
- **Lupus panniculitis (lupus profundus):** Shows lobular involvement with hyaline necrosis of fat, lymphoplasmacytic infiltrates, and possible germinal center formation within lymphoid aggregates.

Treatment

There is no specific treatment for postirradiation sclerodermatous panniculitis. Indurated nodules tend to improve spontaneously over time.

Disorders Erroneously Considered as Specific Variants of Panniculitis

Subcutaneous Lymphomas

(See Chap. 146)

Weber-Christian Disease

Weber-Christian disease was historically used to describe cases of predominantly lobular panniculitis without vasculitis, accompanied by systemic features such as fever and visceral fat involvement. Synonyms included: - Idiopathic nodular panniculitis

- Relapsing febrile non-suppurative nodular panniculitis

However, many cases originally labeled as Weber-Christian disease have since been reclassified with the recognition of distinct entities such as: - **Erythema induratum of Bazin**

- **Pancreatic panniculitis**

- **α_1 -antitrypsin-deficiency panniculitis**

Due to this reclassification, the term *Weber-Christian disease* is now considered obsolete by most authors. It is recommended to abandon this diagnosis in favor of more specific etiologic classifications whenever possible.